

Limye' Foundation

The Complete Service & Operations Blueprint

Version 4.0

<div class="page-break"></div>

Table of Contents

- **Section 1: Executive Summary**
- **Section 2: Organization Overview and Mission**
 - 2.1 Foundation Identity
 - 2.2 Mission and Vision
 - 2.3 Core Values
 - 2.4 Core Service Categories
- **Section 3: Comprehensive Market Analysis**
 - 3.1 Alabama Procurement Landscape
 - 3.2 Detailed Contract Opportunities Analysis
 - A. Inmate Meal Services
 - B. Senior Meal Services
 - C. Youth Services Contracts
 - D. Educational Support Services
 - E. Medical and Mental Health Services
 - F. Community Development and Infrastructure
 - G. Military and Federal Opportunities
 - 3.3 Specialized Market Analysis: Veteran Services
 - A. The VA as a Primary Payer & The Community Care Network (CCN)
 - B. Market Opportunity: NEMT
 - C. Market Opportunity: Medically Tailored Meals (MTM)
 - D. Market Opportunity: Highly Rural Counties & HRTG
 - 3.4 Specialized Market Analysis: Medicare & Medicaid
 - A. Overview: The Alabama Medicaid Agency and CMS
 - B. Medicaid NEMT
 - C. Medicaid HCBS Waivers
 - D. Medicare SDOH Reimbursement & Medicare Advantage
- **Section 4: Integrated Service Delivery Model**
 - 4.1 The Integrated Service Hub Framework
 - 4.2 Core Service Program Details

- 4.3 Keystone Initiative: "Operation Home Base" for Veterans
- 4.4 Limye' Families - A Lifespan Approach
- **Section 5: Organizational Structure, Staffing, & Workforce Strategy**
 - 5.1 Governance and Leadership Structure
 - 5.2 Organizational Chart & Key Roles
 - 5.3 Strategic Sustainability: For-Profit Subsidiary Structure
 - 5.4 **The Limye' NEMT Workforce Advantage: A Strategic Plan**
 - A. Redefining the Role: From Driver to Community Health Transport Specialist
 - B. The "Recruit" Strategy: Building a Purpose-Driven Talent Pipeline
 - C. The "Hire" Process: A Funnel for Selecting Excellence and Empathy
 - D. The "Keep" Framework: A Culture of Growth, Recognition, and Shared Success
- **Section 6: Technology and Equipment Infrastructure**
 - 6.1 Core Technology Systems
 - 6.2 Equipment Requirements
- **Section 7: Financial Projections and Strategy**
 - 7.1 Diversified Revenue Model
 - 7.2 Multi-Year Financial Projections
 - 7.3 Initial Investment Requirements
 - 7.4 Grant Funding & Partnership Strategy
 - 7.5 Financial Controls
- **Section 8: Implementation Roadmap**
 - 8.1 Year 1: Foundation Building
 - 8.2 Year 2: Growth and Stabilization
 - 8.3 Years 3-5: Scale, Impact, and Statewide Leadership
- **Section 9: Marketing and Community Engagement**
- **Section 10: Compliance, Risk, and Quality Assurance**
 - 10.1 Compliance and Regulatory Requirements
 - 10.2 Specialized Compliance: VA, Medicare, Medicaid & NEMT Services
 - 10.3 Risk Management and Mitigation
 - 10.4 Quality Assurance and Outcome Measurement
- **Section 11: Procurement Portal Registration Guide**
- **Section 12: Standard Operating Procedures (SOPs) for Operation Home Base**
 - 12.1 Recruitment and Intake Strategy SOP
 - 12.2 Supportive Housing Program ("20/59 Ventures") SOP
 - 12.3 NEMT Services ("Vets-in-Motion") SOP
 - 12.4 Medically Tailored Meal Program ("Plates for Patriots") SOP

- **Section 13: Conclusion**
- **Appendix A: Works Cited**

<div class="page-break"></div>

Section 1: Executive Summary

Limye' Foundation (formerly Porch Light) is a 501(c)(3) non-profit organization strategically positioned to transform the delivery of essential support services across Alabama. This comprehensive blueprint details a multi-faceted, research-backed strategy to address the critical needs of the state's most vulnerable populations—including seniors, at-risk youth, families in crisis, and a special, high-impact focus on our nation's veterans.

The cornerstone of our strategic vision is "**Operation Home Base**," a synergistic initiative designed to address the most critical Social Determinants of Health (SDOH) for veterans through an innovative integration of stable housing, medically tailored nutrition, and essential transportation. This initiative serves as a powerful, replicable model for our broader mission.

Our financial sustainability is anchored by a sophisticated, diversified revenue model that aggressively pursues government contracts, healthcare reimbursements, grants, and strategic partnerships. A primary pillar of this model is becoming a credentialed, in-network provider for the **U.S. Department of Veterans Affairs (VA)** through its Community Care Network (CCN) administrator, **Optum**, as well as for the **Alabama Medicaid Agency** and **Medicare Advantage plans**. This multi-payer approach is further enhanced by a for-profit subsidiary structure, including entities like **Limye' Transportation LLC** and **Limye' Foods LLC**, designed to generate unrestricted revenue that fuels our core non-profit mission.

This document serves as the single, authoritative guide for the Foundation. It consolidates extensive market research, detailed service delivery models, a groundbreaking workforce development plan for our transportation division, granular financial projections, and step-by-step Standard Operating Procedures. It provides a clear, actionable roadmap for establishing Limye' Foundation as a leader in innovative social service delivery, moving beyond siloed programs to create a true ecosystem of support that improves health outcomes, reduces public costs, and illuminates clear pathways to self-sufficiency for those we serve.

<div class="page-break"></div>

Section 2: Organization Overview and Mission

2.1 Foundation Identity

- **Legal Name:** Limye' Foundation
- **Previous Name:** Porch Light
- **Tax Status:** 501(c)(3) Non-Profit Organization
- **Primary Location:** Tuscaloosa, Alabama
- **Service Area:** Statewide Alabama with an initial focus on the West Alabama region.

2.2 Mission and Vision

- **Mission Statement:** Limye' Foundation empowers vulnerable populations, including our nation's veterans, through comprehensive "prep services" that bridge critical gaps in healthcare access, nutrition, housing, and community support, creating pathways to self-sufficiency and improved quality of life.
- **Vision Statement:** To be the trusted catalyst for transformative community support, connecting Alabama's most vulnerable citizens, with a dedicated focus on veterans, to life-changing healthcare, nutrition, and comprehensive wrap-around services.

2.3 Core Values

- **Human Dignity:** Every person deserves respect and quality care.
- **Cultural Humility:** Honoring diverse backgrounds and experiences.
- **Innovation:** Pioneering solutions to complex social challenges.
- **Collaboration:** Building bridges between services and communities.
- **Accountability:** Measuring and maximizing our impact.
- **Service & Sacrifice:** Honoring the unique contributions and needs of veterans and their families.

2.4 Core Service Categories

1. **Healthcare Navigation & Social Determinants of Health (SDOH):** Community Health Worker (CHW) programs, Medicare SDOH risk assessments, care coordination, health literacy, and chronic disease management support.
2. **Nutritional Services:** Medically Tailored Meals (MTM), senior meal programs, institutional food services, and nutrition education.
3. **Transportation Services:** Non-Emergency Medical Transportation (NEMT), wheelchair-accessible transport, general community transportation, and meal delivery logistics.
4. **Educational & Youth Services:** Youth diversion programs, after-school enrichment, life skills training, and career readiness.

5. **Housing & Community Support:** Elevated in prominence with the new veteran initiative, this includes transitional housing assistance, rapid rehousing, homelessness prevention, family resource centers, and benefits enrollment assistance.
6. **Institutional Services:** Correctional facility support, group home services, and school system partnerships, with **Military and Veteran Support Services** now established as a primary service line.

<div class="page-break"></div>

Section 3: Comprehensive Market Analysis

3.1 Alabama Procurement Landscape Overview

Alabama's procurement system operates through multiple channels, creating a complex but opportunity-rich environment for a well-prepared non-profit. Understanding this landscape is key to our diversified funding strategy.

- **State-Level Systems:**
 - **Alabama Buys (Alabamabuys.gov):** The state's primary e-procurement system and our main portal for tracking state-level RFPs.
 - **STAARS Vendor Self Service (VSS):** A legacy system still used during the state's transition, which we will also monitor.
 - **Agency-Specific Portals:** Many departments (e.g., DHR, DYS, ADMH) maintain their own procurement sites, requiring direct and regular monitoring.
- **Local Systems:**
 - **BidNet Direct:** A centralized platform for many state and local government opportunities, crucial for identifying county and municipal contracts.
 - **County-Specific Portals:** All 67 counties have their own systems and processes, requiring a targeted approach for our expansion plans.
 - **Municipal Procurement Sites:** City-level opportunities, particularly for services like jail meals or community center programming.

3.2 Detailed Contract Opportunities Analysis

A. Inmate Meal Services

- **Alabama Department of Corrections (ADOC):**
 - **Opportunity:** Large-scale, statewide RFPs (e.g., RFP 2025-02) for comprehensive nutritional services.
 - **Requirements:** High barrier to entry as a prime contractor, including a **\$12,000,000 performance bond** and a \$250,000 bid bond.
 - **Strategy:** Pursue **subcontracting opportunities** with prime contractors.

Primes often need specialized partners for therapeutic diets, religious/cultural meals, and local food sourcing.

- **County and City Jails:**

- **Opportunity:** A more accessible market. Mobile County, the City of Hoover, and Jefferson County regularly issue bids for jail meal services.
- **Key Consideration:** The archaic "Sheriff's Food Bill Law" allows sheriffs to retain unspent food funds, creating unique procurement dynamics that necessitate direct relationship-building with county commissions, as it may limit competitive bidding.

B. Senior Meal Services

- **Alabama Department of Senior Services (ADSS):**

- **Program:** Manages the statewide Elderly Nutrition Program (ENP), serving over 335 senior centers via congregate and home-delivered meal models.
- **Challenge:** The program is typically awarded as a single, long-term contract (current provider: TRIO Community Meals), limiting direct opportunities.

- **Area Agencies on Aging (AAAs):**

- **Opportunity:** These are our primary partnership targets. Alabama's 13 AAAs coordinate local services. The **West Alabama Regional Commission AAA** (serving Tuscaloosa) and others like **Aging South Alabama** and **SARCOA** are constantly seeking providers for meal distribution, senior center operations, transportation, and in-home support.

C. Youth Services Contracts

- **Alabama Department of Youth Services (DYS):**

- **Program:** The Diversion Grants Program serves 56 counties and represents a major funding opportunity. DYS funds programs offering needs assessments, counseling, in-home services, and educational support as alternatives to state custody.

- **Alabama Department of Human Resources (DHR):**

- **Opportunity:** A critical partner. DHR issues numerous RFPs for which Limye' is well-suited:
 - **Intensive In-Home Services (IIHS)** to prevent child removal.
 - **Family Service Centers** (multiple counties seeking vendors, including Tuscaloosa).
 - **SNAP Employment & Training (A-RESET)** (multiple counties seeking vendors, including Tuscaloosa).
 - **Independent Living Programs** for foster youth.

D. Educational Support Services

- **Alabama State Department of Education (ALSDE):**
 - **Opportunity:** Multiple federal programs offer contract opportunities, including **Title IV (21st Century Community Learning Centers)** for after-school enrichment and the **McKinney-Vento Program** for homeless students.
- **CHOOSE Act Alabama:**
 - **Opportunity:** This new Education Savings Account (ESA) program allows 'Limye' to register as an "Education Service Provider" (ESP) and receive direct payments from families for approved "educational therapies" and support services.

E. Medical and Mental Health Services

- **Alabama Department of Mental Health (ADMH):**
 - **Opportunity:** Offers professional service contracts and grants like the **ACDD Innovative Ideas RFP** (up to \$50,000) for non-profits serving the IDD population, particularly in the rural Black Belt.
- **Group Homes:**
 - **Opportunity:** These facilities require contracted services for meals, skills training, and transportation, funded through ADMH and Medicaid waivers.

F. Community Development and Infrastructure

- **Alabama Department of Economic and Community Affairs (ADECA):**
 - **Opportunity:** A key source for larger grants. Programs like the **Community Development Block Grant (CDBG)** and **State Economic and Infrastructure Development (SEID)** are available to 501(c)(3)s for projects including community facility development and workforce initiatives.

G. Military and Federal Opportunities

- **Redstone Arsenal (RSAMCC & RFCU):**
 - **Opportunity:** While direct defense contracting is limited, community grant programs from the **Redstone Arsenal Military and Civilians' Club** and the **Redstone Federal Credit Union** offer targeted funding for organizations serving military families and veterans in specific counties.

3.3 Specialized Market Analysis: Veteran Services

Building on the broad market understanding, a specialized analysis reveals a significant and immediate opportunity within Alabama's veteran community.

A. The VA as a Primary Payer & The Community Care Network (CCN)

The VA MISSION Act of 2018 revolutionized veteran care by creating the Community

Care Network (CCN), allowing veterans to receive VA-funded care from local providers. This positions the VA as a major healthcare and social services contractor.

- **Third-Party Administrator (TPA):** Alabama is in CCN Region 3, which is exclusively managed by **Optum Public Sector Solutions, Inc.** This is a critical fact: **To be paid by the VA for community care services, Limye' Foundation must be credentialed and contracted as an in-network provider with Optum.**
- **Eligibility:** Veterans become eligible for community care based on specific criteria (e.g., wait times >20 days, drive times >30 minutes), at which point the VA issues a referral to an Optum provider. Our entire VA reimbursement strategy hinges on this partnership.

B. Market Opportunity: Non-Emergency Medical Transportation (NEMT)

While the VA has its own transportation programs, significant gaps exist. Research confirms the **Disabled American Veterans (DAV) van service at the Tuscaloosa VA Medical Center is currently suspended.** This documented service failure is not a challenge; it is our single greatest strategic entry point. It allows Limye' to approach the Tuscaloosa VA's Community Care Office with a turnkey, immediate solution to a known, pressing problem, positioning "Vets-in-Motion" as an indispensable partner from day one.

C. Market Opportunity: Medically Tailored Meals (MTM)

The veteran population suffers from disproportionately high rates of chronic, diet-related disease (72%). The VA has responded with a national **"Food is Medicine"** initiative. This is institutional validation of our "Plates" program. We will not be selling a new idea; we will be presenting Limye' as the expert implementation partner ready to help the VA achieve its own strategic goals. Our marketing will use the VA's language of "whole health" and focus on measurable outcomes for veterans.

D. Market Opportunity: Highly Rural Counties & HRTG

To serve veterans in outlying counties like Greene, Hale, and Pickens, the **Highly Rural Transportation Grants (HRTG)** program is the key. It provides up to \$50,000 per eligible county.

- **The Challenge:** Federal regulations restrict direct applications to recognized Veteran Service Organizations (VSOs).
- **The Solution:** Our strategy is not to apply directly but to form a **formal subcontracting partnership** with an established VSO (e.g., American Legion, VFW). The VSO will be the prime applicant and grantee, with Limye' Foundation

serving as the contracted operational arm, providing the vehicles, drivers, and logistics. This collaborative approach is permitted by the grant and is our only viable path to rural expansion.

3.4 Specialized Market Analysis: Medicare & Medicaid

Recognizing that many of our clients, including veterans, are dually eligible for other federal and state benefits, a robust strategy to engage with Medicare and Medicaid is critical to our financial sustainability and service integration.

A. Overview: The Alabama Medicaid Agency and CMS

- **Alabama Medicaid Agency:** As the primary administrator for Medicaid in the state, the agency is the gateway for becoming a credentialed provider for NEMT and HCBS waiver services. Alabama operates a fee-for-service model.
- **Centers for Medicare & Medicaid Services (CMS):** CMS sets national Medicare policy. Recent changes have created groundbreaking opportunities for community-based organizations to be reimbursed for addressing Social Determinants of Health (SDOH).

B. Medicaid Non-Emergency Medical Transportation (NEMT)

This represents a significant and stable revenue opportunity.

- **Provider Enrollment:** The primary contractor for managing NEMT brokerages in Alabama is **LogistiCare (now Modivcare)**. Limye' must become a credentialed transportation provider within Modivcare's network to receive trip assignments and process billing.
- **Strategic Advantage:** By becoming a provider for both VA (via Optum) and Medicaid (via Modivcare), our transportation subsidiary can maximize fleet utilization and diversify revenue.

C. Medicaid HCBS Waivers

Limye's services align directly with several waivers:

- **Elderly and Disabled (E&D) Waiver:** This is the most relevant waiver, providing services like case management and **home-delivered meals**. Limye's "Plates" MTM program can be positioned as a high-quality meal provider for E&D waiver participants.
- **Provider Strategy:** Limye' will seek to become a credentialed HCBS waiver provider through the Alabama Medicaid Agency, allowing us to receive referrals from waiver case managers.

D. Medicare SDOH Reimbursement & Medicare Advantage

This is a new and rapidly evolving frontier.

- **New Medicare Codes (Physician Fee Schedule):** As of January 1, 2024, CMS allows healthcare providers to bill for services that our CHWs can deliver "incident to" a physician's supervision:
 - **G0136:** Administration of an SDOH risk assessment tool.
 - **G0019 & G0022:** Community Health Integration (CHI) services.
- **Medicare Advantage (MA):** MA plans are increasingly offering supplemental benefits for **Medically Tailored Meals** and NEMT.
- **Provider Strategy:** Limye' will contract with local physician practices to provide billable CHW services and will also contract directly with major MA plans (BCBS of Alabama, Humana, etc.) to become an in-network provider for their MTM and transportation benefits.

<div class="page-break"></div>

Section 4: Integrated Service Delivery Model

4.1 The Integrated Service Hub Framework

Limye' Foundation's core innovation is its rejection of siloed services. We will operate as a comprehensive, integrated service hub built on a "no wrong door" philosophy.

1. **Universal Access Point:** Every client interaction begins with a universal intake and comprehensive assessment, ensuring we understand the totality of their needs, not just the one they presented with. This leads to warm, internal handoffs between programs.
2. **Levels of Integration:** We will progress through three levels of integration:
 - **Level 1 (Co-Location):** Shared physical space to foster informal coordination.
 - **Level 2 (Coordination):** Shared assessments and regular case conferencing.
 - **Level 3 (Integration):** Unified case management, blended funding, and shared outcomes.
3. **Technology Infrastructure:** An integrated Client Management System (CRM) is the backbone, allowing for real-time service coordination, automated referrals, and robust outcome tracking.

4.2 Core Service Program Details

- **Program 1: Healthcare Navigation and SDOH Services:** We will deploy CHWs (requiring 160-hour competency-based training) and establish contracts with healthcare providers to bill Medicare for SDOH services (G0136, G0019, G0022), creating an immediate earned revenue stream.

- **Program 2: Comprehensive Nutrition Services:** Our "Plates" initiative will provide Medically Tailored Meals, senior meals, and institutional food services, funded through a mix of healthcare reimbursement (VA, Medicare Advantage, Medicaid Waivers), government contracts (AAAs), and private pay. This requires a commercial kitchen (\$31k-\$54k setup cost) with ServSafe and HACCP compliance.
- **Program 3: Multi-Modal Transportation Services:** Our fleet (initial investment \$120k-\$188k) will provide NEMT for our multi-payer contracts, general community transportation, and meal delivery logistics.
- **Program 4: Youth and Family Services:** We will pursue DYS and DHR contracts to provide diversion programs, intensive in-home support, and after-school enrichment.
- **Program 5: Housing and Community Support:** This program focuses on rapid rehousing, transitional housing, and benefits enrollment, serving as a critical feeder for our other services.

4.3 Keystone Initiative: "Operation Home Base" for Veterans

A. Program Philosophy & Target Population

Operation Home Base is our signature integrated care model, designed to stabilize the lives of veterans by addressing three core pillars of well-being: **Housing, Health (Nutrition), and Access to Care (Transportation)**. The program's synergy is its key innovation: stable housing creates a reliable client base for billable nutrition and transportation services. The target population is honorably discharged veterans and their families in West Alabama at risk of or experiencing homelessness, food insecurity, or who have chronic health conditions.

B. Component 1: Supportive Housing ("20/59 Ventures")

This initiative, named to honor the I-20/59 corridor, secures a dedicated block of high-quality rental units for veterans.

- **Operational Partner:** We will pursue a **Master Lease Agreement** with a proven local developer. **Duckworth-Morris Real Estate** is the primary target due to their large portfolio and Section 8 experience.
- **Model:** Limye' will master lease units and then sublet them to veteran clients. Our case managers provide wraparound support.
- **Sustainability:** This creates a concentrated customer base for our billable services. Affordability will be ensured by leveraging federal **HUD-VASH vouchers**.

C. Component 2: Veteran Nutrition ("Plates for Patriots")

This is a specialized track of our "Plates" MTM program.

- **VA Partnership Pathway:**

1. **Direct Referrals:** Establish formal referral pathways with social workers and dietitians at the Tuscaloosa and Birmingham VAs.
2. **CCN Contract:** Aggressively pursue an **Optum contract** to become a credentialed, billable MTM provider.
3. **Pilot Program:** Propose a formal pilot to the Tuscaloosa VA, leveraging their "Food is Medicine" initiative.

D. Component 3: Veteran Transportation ("Vets-in-Motion")

This program operates on three tiers:

1. **Direct-to-VA NEMT:** Our core billable service, transporting veterans to appointments and billing the VA via our **Optum contract**.
2. **Rural County Access (HRTG):** We will operate as a subcontractor to a VSO partner to utilize HRTG funds for serving highly rural counties.
3. **Scheduled Inter-City Routes (Long-Term Vision):** We will work with VA schedulers to establish pre-scheduled "Limye' Days" for high-volume transport between Tuscaloosa and Birmingham to maximize efficiency.

4.4 Limye' Families - A Lifespan Approach

Our model incorporates intergenerational programming to support individuals at every stage of life.

- **Children (0-12):** Early childhood development, after-school programs.
- **Adolescents (13-18):** College prep, career exploration, leadership development.
- **Young Adults (19-25):** Job placement, financial literacy, housing assistance.
- **Adults (26-59):** Career advancement, health management, caregiver support.
- **Seniors (60+):** Healthy aging programs, social connections, intergenerational activities.

<div class="page-break"></div>

Section 5: Organizational Structure, Staffing, & Workforce Strategy

5.1 Governance and Leadership Structure

- **Board of Directors:** A 7-11 member board will provide strategic oversight through committees for Finance, Program, Development, and Governance.
- **Advisory Council:** Comprised of community leaders, service recipients, and subject matter experts.

- **Executive Leadership:** A C-suite structure (CEO, CFO, COO, Chief Development Officer).

5.2 Organizational Chart & Key Roles

- **Department Directors:** Directors will oversee each core service area: Healthcare, Nutrition, Transportation, Youth & Family, Community Support, and a new **Director of Veteran Services**.
- **Direct Service Staff:** Includes Community Health Workers, NEMT Drivers, Kitchen Staff, Case Managers, Youth Specialists, and Benefits Counselors.
- **Support Staff:** Includes HR, IT, Grant Writers, and Communications.

5.3 Strategic Sustainability: For-Profit Subsidiary Structure

To generate unrestricted revenue, a for-profit subsidiary structure is essential.

- **Limye' Transportation LLC:** Houses "Vets-in-Motion," allowing for diversification into private-pay contracts.
- **Limye' Foods LLC:** Operates "Plates for Patriots," enabling catering and public meal prep services.
- **Other potential subsidiaries:** Limye' Properties LLC (real estate), Limye' Logistics LLC (supply chain), Limye' Tech Solutions LLC (IT consulting), and the Algoinsu Insurance Brokerage.

5.4 The Limye' NEMT Workforce Advantage: A Strategic Plan

The NEMT industry is challenged by high turnover. Our strategy is to create a mission-driven workforce advantage by re-envisioning our staff not as "drivers," but as "**Community Health Transport Specialists**."

A. Redefining the Role: From Driver to Community Health Transport Specialist

This strategic re-framing is designed to attract candidates motivated by purpose. It is operationalized through a detailed **Core Competency Model** that translates our values (Human Dignity, Cultural Humility) into observable behaviors (Empathetic Communication, Client-Centered Sensitivity).

Table 5.4.1: The Limye' NEMT Driver Competency Model | Competency Domain | Specific Competency | Behavioral Indicators | Alignment with Limye' Values | | :--- | :--- | :--- | :--- | | **Safety & Compliance** | Defensive Driving & Vehicle Safety | Consistently adheres to traffic laws and company safety protocols. Performs pre- and post-trip vehicle inspections meticulously. Maintains a clean driving record. | Accountability | | HIPAA & Confidentiality Adherence | Protects all client information (verbal, written, electronic). Avoids discussing client details in public or with

unauthorized individuals. | Human Dignity, Accountability | | ADA & Passenger
 Securement | Correctly operates wheelchair lifts and securement systems. Provides
 assistance in a manner that respects passenger autonomy and safety. | Human Dignity
 | | **Client Interaction & Care** | Empathetic Communication | Actively listens to client
 concerns. Maintains a calm, patient, and reassuring tone. Uses clear, simple language.
 | Human Dignity, Innovation | | Client-Centered Sensitivity & Cultural Humility |
 Demonstrates respect for diverse backgrounds. Adapts communication style to meet
 individual needs. Avoids making assumptions. | Human Dignity, Cultural Humility | |
 Trauma-Informed Awareness | Recognizes that clients may have trauma histories.
 Interacts in a way that promotes a sense of safety. Responds to agitation with
 patience and de-escalation. | Human Dignity | | **Operational Excellence** | Punctuality
 & Reliability | Arrives on time for all scheduled pickups. Communicates proactively
 about potential delays. | Accountability | | Route Optimization & Time Management |
 Utilizes GPS and dispatch software effectively. Plans routes to account for traffic and
 appointment times. | Accountability, Innovation | | Proactive Problem-Solving |
 Identifies potential issues (e.g., road closures, client distress) and takes initiative to
 resolve them. Communicates effectively with dispatch. | Innovation | |
Professionalism & Values | Team Collaboration | Communicates respectfully with
 dispatchers, managers, and peers. Contributes to a positive team environment. |
 Collaboration | | Mission Alignment & Professional Conduct | Articulates an
 understanding of the Limye' mission. Maintains a professional appearance and
 demeanor. Acts as a positive ambassador for the organization. | Human Dignity,
 Accountability |

B. The "Recruit" Strategy: Building a Purpose-Driven Talent Pipeline

Our strategy is one of deep community integration.

- **Employee Value Proposition (EVP):** Our recruitment message is: "Start a career on the front lines of community health. Be the vital link that connects vulnerable neighbors to life-changing care. At Limye' Foundation, your work is more than a ride—it's a lifeline."
- **Community-Centric Sourcing:**
 - **Educational Partnerships:** Formal, paid internships and curriculum partnerships with **Shelton State Community College** and the **University of Alabama**.
 - **Veterans' Outreach:** Direct partnerships with **Wounded Warrior Project (WWP)**, **RecruitMilitary**, and local VSO posts.
 - **Faith-Based & Community Networks:** Actively recruit from the volunteer pools of the **Tuscaloosa Ministerial Alliance**.

- **Incentivized Referral Program:** A two-stage financial bonus for employees who refer successful candidates.

C. The "Hire" Process: A Funnel for Selecting Excellence and Empathy

Our multi-stage hiring process assesses both technical skill and core values.

1. **Stage 1: Application & Realistic Job Preview (RJP):** A mandatory RJP video shows candidates "a day in the life" to allow for self-selection.
2. **Stage 2: Competency Assessment:** A behavioral interview with the Hiring Manager uses the **STAR method** and **Situational Judgment Tests (SJTs)**.
3. **Stage 3: Cultural Alignment:** A final interview with the Transportation Manager and a peer uses values-based questions.
4. **Stage 4: Onboarding for Mission Immersion:** A structured 90-day process focused on culture and mentored learning.

D. The "Keep" Framework: A Culture of Growth, Recognition, and Shared Success

- **Total Rewards Reimagined:** Competitive wage plus high-value benefits like a **Lifestyle Spending Account (LSA)** (\$500 annual stipend for car repairs or childcare), paid volunteer time, and a wellness reimbursement.
- **Elite Training Pathway:** A fully-funded, tiered certification path with pay increases at each level:
 - **Tier 1 (Core):** NEMTAC CTS®, PASS, CPR/First Aid.
 - **Tier 2 (Advanced):** Mental Health First Aid (MHFA) certification.
 - **Tier 3 (Master):** Trauma-Informed Care (TIC) and Dementia-Friendly Transportation training.
- **The Limye' Career Lattice:** A flexible model providing multi-directional growth. A Transport Specialist can move vertically (Lead Driver -> Dispatcher) or laterally into other roles (CHW, Family Support Worker).
- **Gainsharing Advantage:** Leveraging **Limye' Transportation LLC**, we will implement a true gainsharing program. A portion of the LLC's quarterly net operating income will be distributed to eligible staff as a cash bonus.

<div class="page-break"></div>

Section 6: Technology and Equipment Infrastructure

6.1 Core Technology Systems

- **AI-Driven Management System:** A robust CRM platform (e.g., Salesforce, HubSpot) is non-negotiable for integrated client tracking, service coordination, and outcome measurement. Annual cost: **\$9,000-\$36,000**.
- **GPS Tracking and Fleet Management:** A system like Verizon Connect or

Samsara is required for NEMT contract compliance, providing real-time tracking, route optimization, and driver behavior monitoring. Annual cost: **\$3,100-\$4,500**.

- **Electronic Health Records Interface:** A HIPAA-compliant system with integration capabilities to securely exchange data with healthcare partners.
- **Financial Management:** QuickBooks Nonprofit with grant tracking and multi-entity accounting capabilities.

6.2 Equipment Requirements

- **Vehicle Fleet (Year 1):**
 - 1-2 Wheelchair Accessible Vehicles (WAVs): ~\$65,000-\$98,000 each
 - 1-2 Standard Medical Transport Vehicles: ~\$15,000-\$25,000 each
 - 1 Cargo Van (For Meal Delivery): ~\$25,000-\$40,000
 - **Total Initial Fleet Investment: \$120,000 - \$188,000**
- **Commercial Kitchen Setup:**
 - Estimated cost for industrial refrigeration, ranges, and prep equipment: **\$31,000 - \$54,000**.
- **Office & Program Equipment:**
 - Estimated cost for furniture, computers, and supplies: **\$15,000 - \$30,000+.**

<div class="page-break"></div>

Section 7: Financial Projections and Strategy

7.1 Diversified Revenue Model

Our financial stability is predicated on a diversified mix of revenue streams:

1. **Government Contracts (40-45%)**
2. **Healthcare Reimbursement (25-30%)**
3. **Grants (15-20%)**
4. **For-Profit Subsidiaries (5-10%)**
5. **Private Support (3-5%)**

7.2 Multi-Year Financial Projections

- **Year 1 Projections:**
 - **Revenue (Low-High):** \$200,000 - \$375,000
 - **Operating Expenses (Low-High):** \$355,000 - \$625,000
 - **Net Loss (Low-High):** (\$155,000) - (\$250,000)
- **Year 3 Revenue Target: \$1,500,000**
- **Year 5 Revenue Target: \$3,000,000**

7.3 Initial Investment Requirements

Category	Low Estimate	High Estimate
Legal and Formation	\$8,600	\$17,500
Technology Systems	\$15,000	\$25,000
Office Setup	\$10,000	\$20,000
Vehicle Fleet (Revised)	\$120,000	\$188,000
Equipment	\$50,000	\$100,000
Training	\$18,500	\$68,500
Working Capital	\$50,000	\$100,000
Total Initial Investment	\$272,100	\$519,000

7.4 Grant Funding & Partnership Strategy

Funder Name	Focus Areas	Grant Range	Application Timing
Community Foundation of West Alabama	Health, human services, education	\$5,000-\$50,000	Rolling basis
Alabama Power Foundation	Community development, education	\$10,000-\$100,000	Quarterly
The Caring Foundation (BCBSAL)	Healthcare access, wellness	\$25,000-\$150,000	Annual cycle
The Daniel Foundation of Alabama	Rural health, aging services	\$50,000-\$500,000	Bi-annual
Children's Trust Fund of Alabama	Child abuse prevention	\$15,000-\$75,000	Annual
ADECA	Community development, housing	\$100,000 - \$1,000,000+	Annual NOFA

7.5 Financial Controls

- **Internal Controls:** Strict segregation of duties, monthly board-level financial review, and an annual external audit.
- **Compliance:** Adherence to all IRS Form 990 filing requirements, A-133 Single Audit (if applicable), and all grant/contract financial reporting.

<div class="page-break"></div>

Section 8: Implementation Roadmap

8.1 Year 1: Foundation Building (Months 1-12)

- **Phase 1: Legal & Structural Formation (Months 1-3 | Cost: \$8.6k-\$17.5k):** Complete 501(c)(3) incorporation, state/local registrations, establish bank accounts and financial policies.
- **Phase 2: Infrastructure & Partnership Development (Months 4-6 | Cost: \$58k-\$100k):** Secure office space, select and implement technology systems (CRM, Fleet Management). Begin staff recruitment and initiate formal partnership discussions with the VA, Duckworth-Morris, and key healthcare providers.
- **Phase 3: Program Launch & Initial Service Delivery (Months 7-9 | Cost: \$170k-\$288k):** Hire and train the initial cohort of staff. Procure vehicles and equipment. Launch the first services (e.g., CHW, initial NEMT routes).
- **Phase 4: Evaluation & Expansion Planning (Months 10-12 | Cost: \$50k):** Evaluate initial program performance against KPIs. Submit major grant applications (DHR, DYS, HRTG). Finalize for-profit subsidiary structures.

8.2 Year 2: Growth and Stabilization

- Expand services to a second county.
- Launch the full "Plates for Patriots" nutrition program.
- Achieve full credentialing and begin billing Medicaid and Medicare Advantage.
- Launch for-profit subsidiaries and begin generating unrestricted revenue.

8.3 Years 3-5: Scale, Impact, and Statewide Leadership

- Achieve a 5-county service area and the \$1.5M revenue target by Year 3.
- Expand to 10+ county coverage and reach the \$3M revenue target by Year 5.
- Achieve national accreditation (e.g., NCQA).
- Establish an endowment to ensure long-term sustainability.
- Begin developing a replication model to share our success with other communities.

<div class="page-break"></div>

Section 9: Marketing and Community Engagement

Brand Positioning

- **Brand Promise:** "Illuminating Pathways to Possibility"
- **Core Messages:** Comprehensive support under one roof, culturally responsive services, evidence-based approaches, community-driven solutions, measurable impact.

Target Audiences

- **Primary:** Service recipients, referral partners (healthcare systems, government agencies).
- **Secondary:** Donors, funders, corporate partners, volunteers.
- **Tertiary:** Policymakers, academic institutions, general public.

Marketing Channels

- **Digital Strategy:** Professional website, social media, email marketing, online referral portal.
- **Traditional Outreach:** Community presentations, health fairs, print materials, radio partnerships.
- **Professional Networking:** Conference presentations, coalition participation.

Community Engagement Plan

- **Stakeholder Engagement:** Quarterly community forums, advisory council meetings, client feedback sessions.
- **Cultural Competency:** Multilingual materials, cultural liaisons, inclusive programming.
- **Volunteer Program:** Structured recruitment, training, and recognition system.

<div class="page-break"></div>

Section 10: Compliance, Risk, and Quality Assurance

10.1 Compliance and Regulatory Requirements

The Foundation will maintain strict adherence to all applicable regulations:

- **Federal:** IRS 501(c)(3) rules, HIPAA, Stark Law, Anti-kickback statute.
- **State:** Alabama charitable organization registration, professional licensing, vehicle regulations.
- **Program-Specific:** DOT/ADA regulations for transport; Health department

permits for food service; mandatory reporting laws for youth services.

- **Insurance:** A comprehensive insurance portfolio is required, with an estimated annual budget of **\$60,000 - \$120,000**. Coverage will include General Liability, Professional Liability, Auto Liability (\$1M+), Property, Workers Compensation, Directors & Officers (\$1M), Cyber Liability, and Employment Practices.

10.2 Specialized Compliance: VA, Medicare, Medicaid & NEMT Services

This is a high-risk, high-reward area requiring meticulous attention to detail.

- **VA CCN Provider Agreement:** We must meticulously adhere to all terms of our network provider agreement with Optum.
- **HealthShare Referral Manager (HSRM):** All relevant staff must be fully trained and proficient on the VA's mandatory HSRM system for receiving referrals.
- **Medicaid Broker Agreement:** We must become a credentialed transportation provider with Modivcare to receive Medicaid NEMT referrals.
- **NEMT Provider Requirements:** The "Vets-in-Motion" program must meet the VA's stringent requirements:
 - **Insurance:** Tiered commercial auto liability insurance projected to be at least **\$350,000** for small vehicles and **\$1,500,000** for larger vans.
 - **Vehicle Standards:** All vehicles must be ADA-compliant with daily inspection logs.
 - **Driver Qualifications:** All drivers must pass extensive background checks and maintain certifications in CPR, First Aid, and Defensive Driving.

10.3 Risk Management and Mitigation

A formal risk management framework will be implemented: | Risk Category | Specific Risk | Mitigation Strategy | | :--- | :--- | :--- | | **Strategic** | Funding Concentration | Diversify revenue streams; target no single contract > 30% of revenue. | | **Operational** | Workforce Turnover | Implement the comprehensive "Recruit, Hire, Keep" strategy (Sec 5.4). | | **Financial** | Cash Flow Delays | Secure a line of credit; maintain a 6-month operating reserve. | | **Reputational** | Service Failure / Data Breach | Implement robust QA, have a rapid incident response plan, maintain cyber liability insurance. |

10.4 Quality Assurance and Outcome Measurement

- **Performance Framework:** We will track Operational KPIs (Client Satisfaction >90%), Program KPIs (Reduced hospital readmissions), and Financial KPIs (Revenue growth >25% annually).
- **Data Management:** Our integrated CRM will provide real-time dashboards for daily, weekly, and monthly reporting.
- **Continuous Improvement:** A process of monthly data reviews and quarterly

improvement cycles will be implemented.

<div class="page-break"></div>

Section 11: Procurement Portal Registration Guide

A systematic approach will be taken to register with all necessary procurement portals.

- **State-Level:**
 1. **Alabama Secretary of State:** For incorporation. (Cost: \$100-\$200)
 2. **Alabama Attorney General (Charitable Organizations):** Required for non-profits. (Cost: \$25 annually)
 3. **E-Verify System:** Required for all state/local contracts. (No cost)
 4. **Alabama Buys:** Primary state portal. (No cost)
 5. **STAARS Vendor Self Service:** Legacy state portal. (No cost)
- **Multi-Jurisdictional:**
 - **BidNet Direct:** Covers multiple state and local governments. (Free for limited access)
- **County-Specific (Priority):**
 - Jefferson County, Mobile County, Madison County, Tuscaloosa County, Montgomery County.
- **Federal:**
 1. **SAM.gov (System for Award Management):** Required for all federal contracts and grants. (No cost)
 2. **Grants.gov:** Required for federal grant applications. (No cost)

<div class="page-break"></div>

Section 12: Standard Operating Procedures (SOPs) for Operation Home Base

12.1 Recruitment and Intake Strategy SOP

1. **Outreach:** The Director of Veteran Services will establish formal MOUs with the Tuscaloosa and Birmingham VA Medical Centers (targeting Social Work and Community Care offices) and engage local VSO posts.
2. **Intake Process:**
 - **Screening:** An Intake Coordinator confirms veteran status and need.
 - **Benefits Verification:** A Case Manager assists with VA healthcare enrollment (VA Form 10-10EZ) if necessary.
 - **Assessment:** A comprehensive needs assessment is administered.

- **Service Planning:** An Individualized Service Plan (ISP) is collaboratively developed, explicitly outlining engagement in housing, nutrition, and transport.

12.2 Supportive Housing Program ("20/59 Ventures") SOP

1. **Eligibility:** Veteran must be enrolled in Operation Home Base with a signed ISP. Priority is given to homeless/at-risk veterans.
2. **Placement:** Case Manager assists with the sublease application and paperwork for housing subsidies (HUD-VASH), coordinating move-in with the property partner.
3. **Case Management:** Regular, in-person check-ins are conducted (weekly for the first month, then tapering) to ensure connection to services and mediate with property management to prevent eviction.

12.3 NEMT Services ("Vets-in-Motion") SOP

1. **Booking (CCN Referrals):** A VA referral/authorization is received via the HSRM portal. The dispatcher verifies the authorization, books the ride in the fleet software, and assigns a driver.
2. **Efficiency Strategy:** The director works with VA schedulers to create "Limye' Days" for high-volume transport from specific locations (like the 20/59 housing cluster), allowing for the efficient use of multi-passenger vans.
3. **Trip Execution:** Drivers conduct pre-trip safety checks, provide courteous and professional curb-to-curb assistance per ADA standards, and confirm return trip procedures.
4. **Post-Trip & Billing:** The driver logs trip data in real-time. Administrative staff reconciles logs and submits electronic claims to Optum for payment.

12.4 Medically Tailored Meal Program ("Plates for Patriots") SOP

1. **Enrollment:** A veteran is identified via intake or a direct VA provider referral, which must include a qualifying diagnosis and dietary prescription. Limye's RD assigns the appropriate meal plan.
2. **Production & Delivery:** The kitchen prepares meals adhering to RD-approved recipes. Meals are packaged, labeled, and delivered via optimized routes, focusing on the "20/59 Ventures" housing cluster.
3. **Billing & Reporting:** Claims are submitted to Optum for CCN referrals. Data is tracked for grant reporting. The RD provides quarterly summary reports to referring VA providers, closing the care loop.

<div class="page-break"></div>

Section 13: Conclusion

The integration of the "Operation Home Base" initiative into the Limye' Foundation's comprehensive service model represents a pivotal evolution for the organization. This blueprint provides a detailed, actionable, and research-backed strategy to address the complex, interconnected needs of Alabama's most vulnerable populations, with a special emphasis on our veterans.

By moving beyond siloed services to create a synergistic ecosystem of housing, nutrition, and transportation, funded by a diversified portfolio of public and private payers, the Foundation is positioned to deliver transformative, measurable outcomes. The plan acknowledges and provides clear pathways to navigate the complexities of partnering with the Department of Veterans Affairs, the Alabama Medicaid Agency, and Medicare Advantage plans. The detailed Standard Operating Procedures and the groundbreaking workforce development plan provide the granular, on-the-ground guidance necessary for successful implementation, risk mitigation, and the establishment of a culture of excellence.

Successful execution of this plan will not only provide life-changing support to thousands of Alabamians but will also establish Limye' Foundation as a leader in innovative social service delivery. It creates a robust, financially sustainable model that leverages diverse revenue streams to fuel its mission. This initiative will illuminate a new pathway to possibility, not just for the clients we serve, but for the future of the Foundation itself. The need is clear, the pathway is illuminated, and the time for action is now.

<div class="page-break"></div>

Appendix A: Works Cited

This blueprint is a synthesis of extensive research and several foundational internal documents. The primary sources informing this comprehensive plan include:

- **Limye' Foundation Comprehensive Service Blueprint v10:** Provided the exhaustive initial market analysis of statewide contract opportunities.
- **Limye' Foundation Blueprint v9:** Served as the initial condensed strategic framework.
- **Veterans' Services Business Plan Expansion:** Provided the detailed research and strategic focus for the "Operation Home Base" initiative, including analysis of the VA, Optum, and HRTG.
- **The Limye' NEMT Workforce Advantage:** Supplied the complete, in-depth

strategy for recruiting, hiring, and retaining a mission-driven transportation workforce.

A complete, dynamic, and hyperlinked list of all individual data points and web-based citations is maintained in a separate internal document for operational reference.

```
<script> function setPageNumbers() { const pageFooters =  
document.querySelectorAll('.page-footer'); for (let i = 0; i < pageFooters.length; i++) {  
pageFooters[i].innerHTML = Page ${i + 1}; } // This is a simplified script for  
demonstration. // In a true print scenario, a more robust script or CSS would be  
needed. const pages = document.body.innerHTML.split('<div  
class="page-break"></div>'); const footer = document.getElementById('page-footer');  
if (footer) { footer.innerHTML = Page 1 of ${pages.length}; } } window.onload =  
setPageNumbers; </script>
```